

**JUSTIFICATION FOR A SOLE SOURCE AWARD
FOR CERTAIN COMMERCIAL ITEMS**

1. Contracting Activity: Department of Veterans Affairs
Office of Procurement, Acquisition, and Logistics
Technology Acquisition Center
23 Christopher Way
Eatontown, NJ 07724

2. Description of Action:

The proposed sole source action is for a firm-fixed price (FFP) contract for Care Transitions Portal Software Licenses with Engineered Care, Inc, 44 Montgomery ST STE 3500, San Francisco, California 94104-4808.

3. Description of Supplies or Services:

The proposed action is to provide the 6 Veteran Affairs Medical Centers (VAMCs) within VISN 6 access to the Care Transitions Software and Maintenance Support application (the Care Transitions Portal) to create a patient friendly post-discharge handbook ("After Hospital Care Plan booklet") that summarizes their health status, next appointments, medications, diagnoses, etc. The discharge handbook is created from information in the VA's Computerized Patient Record System (CPRS) and from the VA's Pharmacy package. The Care Transitions Portal is used by VAMCs around the country, and has been used in VISN 6 for the past 3 years. The contractor will ensure that the system properly interfaces with CPRS and will provide as-needed training to staff on its use. The total estimated value of the proposed action is [REDACTED] (inclusive of options).

4. Statutory Authority:

The statutory authority permitting other than full and open competition is 41 U.S.C. 1901 as implemented by the Revolutionary FAR Overhaul (RFO) 12.201-1, entitled "Simplified Procedures".

5. Rationale Supporting Use of Authority Cited Above:

Based on the market research, as described in section 8 of this justification, it was determined that only one company is viable for the required Care Transitions Portal product. The Care Transitions Portal product, "After Hospital Care Plan booklet" was created with VA's Office of Information and Technology and is designed to interface with the VA's Electronic Health Record, "Computerized Patient Record System," (CPRS) and with "Veterans Health Information Systems and Technology Architecture" (VISTA) to create a handbook to give to patients at their discharge from an inpatient stay. The software must be able to extract patient specific data including medications (name, dose, schedule, instructions), upcoming appointments, physician information, etc. This information must be extracted from CPRS/VISTA to

eliminate manual data entry and the chance for errors. Further, this information must be presented at a reading and comprehension level that is appropriate for veterans. Engineered Care, Inc. has successfully implemented the software at more than 25% of VAMCs Nationwide and has demonstrated that the Care Transitions Portal can extract and compile accurate data from CPRS and VISTA. The software has already been approved by and meets all requirements of the VA's Information Security Office and Privacy Office. No other software has been revealed to be approved for the functions required in this requirement. The software is currently housed on a server that is part of the VA Region 3 Data Center in Orlando, FL, and meets all security requirements. Without the Care Transitions Portal, patients may experience adverse clinical outcomes including poor adherence to new medication regimens, poor attendance at post discharge follow up clinic visits, and a higher risk of readmission. Data from the Agency for Healthcare Research and Quality (AHRQ) and from NIH studies indicate that the average cost of a readmission is \$15,000. In FY25, among the 7 sites in VISN 6, there were 2,260 readmissions. If this number could be reduced by as little as 10%, \$3.4M in readmission costs may be avoided (226 readmissions * \$15,000).

Because VISN 6 has already been using the Care Transition Portal software to create the After Hospital Care Plan booklet for the last 3 years, we have sunk costs associated with launching the product, training staff at our sites, and customizing (and maintaining) the booklet for our sites and discharging services (Medicine, Surgery, etc.). In our initial contract \$35,000 was earmarked for on-site training, but this does not take into account the hours of staff time spent working with the contractor to obtain the specific notes titles and file locations that were needed to populate the After Hospital Care Plan booklet, provide initial review and quality control, pilot test the booklet in a specific ward for usability, and work with the contractor to correct any issues or provide any requests for optimization. Estimates from sites indicated that a proper implementation, including content set-up, testing, training of additional staff, etc., required 25% efforts from a provider, a mid-level provider, and a program analyst over the course of a year. The annual salary costs for these 3 employees totals \$525,000, not including benefits, and a 25% effort equates to \$131,250 per site. Were VISN 6 to engage a new product and contractor we would expect to have to invest these resources again to get the new software in place and operating to the same level as Engineered Care's Care Transitions Portal. Multiplied by the 6 interested VA Medical Centers, we could expect salary costs of approximately \$787,500 to establish new software, as well as VISN COR and program management costs of approximately \$15,000. During the time that any new software was being put in place, sites would not have a tool to use to communicate discharge instructions to patients in one easy, cohesive format, and VISN 6 would either have to seek a bridge contract with Engineered Care while we operationalized the new contract, or would have to return to previous approaches used prior to 2023, which would also require training, and which would not be as seamless or personalized, and which would result in a fragmented and frustrating process for staff.

6. Efforts to Obtain Competition:

Market research was conducted, details of which are in the market research section of this document. This effort did not yield any additional sources that can meet the Government's requirements. Additionally, the proposed action will be synopsized on the Contracting Opportunities Page in accordance with RFO 5.201. Any proposals that are received shall be evaluated.

7. Actions to Increase Competition:

The Government will continue to conduct market research to ascertain if there are changes in the marketplace that would enable future actions to be competed.

8. Market Research:

The Government's technical experts conducted internet-based research in January 2026 to determine if other brand items could meet VA requirements and be compatible with CPRS and VHA's current Electronic Health Record. No such products were found.

The Technology Acquisition Center (TAC) issued a Request for Information (RFI) on February 3, 2026, via a RFI posting to the Contracting Opportunities page (within Sam.gov) to ascertain if there were any other sources that could meet the Government's requirements. RFI 36C10B26Q0173 closed on February 9, 2026, with no responses received.

Additional market research was conducted by the TAC in February 2026 by utilizing the NASA SEWP V Provider Lookup Tool. The search using keywords "Care Transition Portal" and "Engineered Care" identified 0 contract holders. Based on the above, there is no reasonable expectation that this product exists on the National Aeronautics and Space Administration (NASA) Solutions for Enterprise-Wide Procurement (SEWP) V Government-wide Acquisition Contract (GWAC). Lastly, the TAC utilized the General Services Administration (GSA) e-library tool searching the same keywords. No results were found in the search and therefore there is no reasonable expectation that this product exists on the GSA GWAC.

As a result of the aforementioned market research, there is no reasonable expectation that any other vendor besides Engineered Care can provide for the need.

9. Other Facts:

None

10. Technical and Requirements Certification:

I certify that the supporting data under my cognizance, which are included in this justification, are accurate and complete to the best of my knowledge and belief.

[Redacted Signature]

Ms. Katherine McMullin
Clinical Informatics and Operations Supervisor
VISN 6
Department of Veterans Affairs

[Redacted Signature]

Date

11. Fair and Reasonable Cost Determination:

I hereby determine that the anticipated price to the Government for this contract action will be fair and reasonable consistent with the price analysis techniques held within RFO 12.204(a).

[Redacted Signature]

Ms. Wendella Baptiste
Contracting Officer
Technology Acquisition Center
Office of Procurement, Acquisition, and Logistics
Department of Veterans Affairs

[Redacted Signature]

Date

12. Procuring Contracting Officer Certification/Approval:

I certify that this justification is accurate and complete to the best of my knowledge and belief. As this contract action exceeds the simplified acquisition threshold, but does not exceed \$900,000, the certification required by RFO 6.104-1(a)(12) serves as approval.

[Redacted Signature]

Ms. Wendella Baptiste
Contracting Officer
Technology Acquisition Center
Office of Procurement, Acquisition, and Logistics
Department of Veterans Affairs

[Redacted Signature]

Date